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Editors

# Human Dignity

Establishing Worth and Seeking Solutions

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## Preface

Defining dignity has taken on its own life force. Scholars including philosophers, lawyers, ethicists, historians and human rights researchers continue to explore the meaning of human dignity from different theoretical perspectives and historical vantages. The concept also has its critics with perhaps the most famous being Arthur Schopenhauer (1965) who claimed dignity was devoid of all meaning and that it was a “shibboleth of all perplexed and empty-headed moralists”.

Nearly all of the commentary about dignity refers to the essential difficulties of definition. For example, Martha Nussbaum (2011) describing the creation of capabilities in the Human Development Approach states that dignity is an “intuitive notion” that is by no means entirely clear. She suggests that if it is used in isolation as if it is completely self-evident, it can be used “capriciously and inconsistently” (p. 32). In her examination of discrimination law, Sandra Fredman (2011) states that while dignity is an “intuitively appealing concept” (p. 20) there are also difficulties with the concept being open to both differing interpretations and even opposite results. She argues that dignity should be considered as one facet of a multidimensional notion of equality, which also accommodates disadvantage, acceptance of difference and participation.

For Jeremy Waldron, dignity is “a sort of status-concept” that has to do with the standing in the sense of formal legal standing of a person and

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**Dr. Susan Banki's** research interests lie in the political, institutional and legal contexts that explain the roots of and solutions to international human rights violations. In particular, she is interested in the ways that questions of sovereignty, citizenship/membership and humanitarian principles have shaped our understanding of and reactions to various transnational phenomena, such as the international human rights regime, international migration and the provision of international aid. Susan's focus is in the Asia-Pacific region, where she has conducted extensive field research in Thailand, Nepal, Bangladesh and Japan on refugee/migrant protection, statelessness and border control. She is currently investigating the local, regional and international mechanisms (and the interactions between them) that serve as potential levers for change.

**Nicole Phillips** completed her Masters in Human Rights at the University of Sydney in 2014. Her main research interests include migrant and refugee rights in South-East Asia, Australia and the United States. Prior to completing her Masters, Nicole studied at Indiana University and the University of Bologna in Italy. Nicole has conducted field research on Bhutanese refugees in Nepal and on young Burmese women in Thailand with Professor Susan Banki and has worked in resettlement in the United States.

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## Human Rights, Transnational Migrations and the Changing Role of Citizenship

Maria Rita Bartolomei

### Preliminary Remarks and Research Methods

The migration of people is a feature of human history. Nowadays, however, immigration appears as a basic structural trait of nearly all industrialized countries (Massey et al. 1993). What is new are the size and origin of immigration, as well as the reasons for emigration: not only poverty, famine, unemployment and the so-called international wage differential but, increasingly, also persecutions, environmental disasters and wars (ICJ 2014, pp. 38ff.). Hence we witness a growing complexity in migration flows. If, for example, the difference between refugees and voluntary immigrants can be clear from a legal point of view (1951 Convention; 1967 Protocol Relating to the Status of Refugees), in reality the reasons to move may be a combination of choice and compulsion. This phenomenon, called “mixed migration flows” (UNHCR 2006), inevitably contributes to making international mobility – particularly

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“illegal” migration – a pressing policy and advocacy issue. In fact, while the right to emigrate is recognized at the international level (Art. 13 UDHR 1948), the decision on who can enter a country and on the acquisition, loss and deprivation of citizenship remains a matter of sovereignty. Therefore, we witness the simultaneous occurrence of two rather contradictory phenomena. On the one hand, nation-states often react by strengthening their frontiers through restrictive border controls and expressions of nationalist ideologies (Geddes 2000). On the other hand, the global expansion and intensification of human rights discourse put nation-states under increasing outside pressure to extend membership rights to aliens (Elliot 2011).

Since at present migration is the major sources of social and legal change and one of the most important topics of both public and academic debate, a large body of literature analysing its various aspects already exists. There have also been several recent theoretical approaches showing an understanding of how the current phase of globalization is transforming and shaping the meaning and the significance of citizenship, especially as far as the role of immigrant is concerned (Aleinikoff and Klusmeyer 2001; Benhabib 2004; IOM, 2004, pp. 3, 3.7; Mantu 2015). Nonetheless, there appears to be a lack of empirical research and of theoretical approaches exploring the relationship between the right of citizenship and some human rights such as, for example, the right to health. With rare exceptions (Biswas et al. 2012; Cuadra 2012), in fact, the study of the access to healthcare by undocumented migrants from a human rights perspective has received almost no attention.

Drawing on current literature on the topics, this chapter aims to explore the complex, multifaceted interrelationships between citizenship, migration and human rights, focusing especially on the right to health (Romero-Oturno 2004). This is an interesting theoretical challenge, if we consider that it is both a social right, which individuals should be entitled to as members of society, and one of the most fundamental rights, closely linked to the rights to life and to physical and mental integrity; and therefore, absolutely critical to individual and collective well-being (Artt. 3, 25 UDHR; Artt. 1–3 EU Charter; General Comment No. 14/2000 of Art. 12 ICESCR; UN Special Rapporteur on the Right to Health Care 2013). Moreover, as migrants put new kinds of

pressure on the welfare state, the scientific debate cannot ignore the fact that deciding who deserves healthcare and, particularly, what kind of healthcare is becoming a major issue (Olafsdottir and Bakhtiari 2015).

My current research includes a number of case studies on European and non-European countries. Here I will consider only the situation of Italy, France, Spain and England (UK)<sup>1</sup> respectively, as these European countries are particularly affected by transnational migration (EUROSTAT 2015) and their debates on citizenship and human rights are particularly intense and passionate.

The research work was carried out at different times,<sup>2</sup> using qualitative methodologies, such as participant observation and ethnographic interviews (294).<sup>3</sup> Though very aware of the complexity and diversity of the migration experience, in order to simplify and speed up my work, I choose to interview the same categories of professionals in every country,<sup>4</sup> but different groups of immigrants, chosen as they seemed rather representative and because I could more easily access them: Nigerians in Italy; Moroccans in France; Syrians in Spain; Pakistani in England. I will use some excerpts from these interviews to illustrate my discussion. I also closely examined vast range of official documents produced by the various governments. I cannot report all my findings here, so I will limit myself to some observations on a few important outcomes. Specifically, I will highlight the fact that, in all the countries considered, the essential aspects of the fundamental right to health are not being fulfilled for undocumented migrants. I would like also to stress how citizenship often not only reflects, but in some cases directly reinforces, boundaries of exclusion and inclusion in a national community.

<sup>1</sup> In the United Kingdom each region (England, Northern Ireland, Scotland and Wales) has different policies and priorities, and thus its own system of publicly funded healthcare, so here I will refer only to England.

<sup>2</sup> In Italy, from January 2013 to December 2014 (Ancona and Ascoli Piceno); in France, March–April 2014 (Lyon); in England, in May–June 2014 (London); in Spain, in May–June 2015 (Madrid).

<sup>3</sup> Respectively: 85 in Italy, 75 in France, 69 in Spain and 65 in England.

<sup>4</sup> Politicians (at national, regional and local levels); doctors; general practitioners; healthcare assistants; clerks at health counters; legal immigrants; undocumented immigrants.

## Citizenship, Human Rights and HealthCare Policy

Since the classical idea of *πολιτεία* voiced by Aristotle (1995), through Marshall's concept of social citizenship (1992), the notion of citizen has evolved over the centuries, with increased opportunity to participate in the process of popular self-governance and a consolidation of his/her identity as a holder of rights. The right of citizenship was developed as a legal framework to indicate both a status and a legal relationship, which determines rights and responsibilities for individuals, States and communities (Smith 2002). In that it is experienced through belonging to a national community with shared memories, values and purposes, citizenship also involves identity (Schwarzmantel 2007). A person can be a citizen for several reasons: *jus sanguinis* (parents are citizens); *jus soli* (born within a country); *jus matrimonii* (marriage to a citizen); naturalization and so on. In the past exclusions were made on grounds such as skin colour, ethnicity, sex and free status. Most of these exclusions no longer apply in most places. Despite this, there may be significant differences in the processes by which states grant citizenship and a differentiation in the category of citizen (Isin and Turner 2002). It follows that the simultaneous application of different criteria may lead to multiple citizenships (Faist and Kivisto 2007). As part of the notion of state sovereignty, states are generally free to establish their own laws and, therefore, who their own citizens are and, from there, the admission, detention, removal or expulsion of migrants. However, although states have the power to manage migration flows into, through and from their territories,<sup>5</sup> yet by international law they must do so in such a way as to uphold the rights of individuals – i.e. human rights – within their territory and under their jurisdiction (ICJ (International Commission of Jurists) 2014).

The concepts of citizenship and human rights share similar roots in liberal individualism and have long been closely linked yet do not coincide (De Sousa Santos 2008; Nash 2009). While the rights and obligations

<sup>5</sup> With the notable exception of the principle of “non-refoulement” (see The 1951 Refugee Convention).

attached to citizenship are granted and determined by internal law, human rights are inalienable, self-legitimizing (do not need democratic deliberation and decision making) and should not be granted on the basis of citizenship. “The concept of human rights is an artifact of modern Western civilization” (Donnelly 1982, p. 303), it attempts to define universal personhood outside of the nation-state framework. Originating first and foremost as a proclamation of moral principles, human rights are becoming increasingly legalized. Their legalization through specific international agreements and their implementation through a special European Court transform both international and domestic law. The Universal Declaration of Human Rights, as a set of prescribed limits to a government's conduct towards people under its jurisdiction, gives individuals human rights and also the responsibility to uphold human rights, regardless of nationality and citizenship status (Art. 2 UDHR).

For the last century, with the introduction of the National Health Service, most Western countries have secured some form of universal healthcare for their citizens, equating the right of being a citizen with the right of getting medical treatment when ill (Kuhlmann et al. 2015). As migrants are not citizens of the receiving states (procedures for eventually obtaining citizenship vary from country to country, but they are always very long and complex), due to their status they are often excluded from national health systems. Nevertheless, to comply with international human rights law, states should ensure the “highest attainable standard of health” (G.C. 14/2000) to any individual, regardless of nationality or migration or residence status. Furthermore, according to this Comment, the right to health has a “core content” referring to a “minimum essential level”,<sup>6</sup> which should be recognized to all people. In practice, however, states often strain to fully implement the right to health of migrants, particularly those in irregular or illegal situations.

For all these reasons, I think it could be interesting to focus on the ways in which undocumented migrants in some countries have their healthcare rights enacted, ignored or suppressed.

<sup>6</sup> It implies essential primary healthcare; minimum essential food; sanitation; safe and drinkable water; essential drugs. <http://www.ohchr.org/EN/Issues/ESCR/Pages/Health.aspx>.



## Italy, France, Spain and England

In this section I will report on a few outcomes of fieldwork I carried out in Italy, France, Spain and England, in order to highlight how, in each country considered, undocumented migrants hardly ever enjoy the “essential aspects” of the fundamental right to health.

According to the abovementioned General Comment (14/2000), the right to health has four key aspects or elements: availability, accessibility, acceptability and quality. In order to make my discussion easier and clearer, I will use these elements as a framework to organize my outcomes. I will therefore not categorize the answers (obtained in interviews) according to the country of respondents, but against these categories. Additionally, I will not report the answers given by all the selected categories of people in each country,<sup>7</sup> but only a few meaningful excerpts. I will develop mainly the dimension of “accessibility” because it is the most complex and important for our discussion; I will report particularly on the outcomes related to Italy, where I spent the longest time in fieldwork and interviews.

### Availability

Until 2012, the Spanish health legislation guaranteed both citizens and noncitizens – including irregular immigrants – the right to public health coverage. Unfortunately, since Law 16/2012 entered into force, its austerity measures have deprived immigrants with irregular status of their previous full access to the public care system.<sup>8</sup>

In response to economic pressure, a combination of concrete sets of measures were devised to transform the public health system, each with a different impact on the availability and quality of the health care services in the country (P.D., Spanish general practitioner, female).

<sup>7</sup> See par. 1, n. 5 and 6. Quotations by politicians do not bear the interviewees’ initials, as they agreed to grant interviews provided they were not identified.

<sup>8</sup> <http://spain.angloinfo.com/healthcare/health-system/benefits/>.

Immigrants are forced to buy private health insurance, especially as the public health system does not cover dental care (T.J., Syrian undocumented immigrant in Spain).

The decision taken in some regions to ignore the ban, however, affected the central government’s policy, and recently Spain’s government has announced that it will allow illegal immigrants to access free public healthcare.<sup>9</sup>

Banning undocumented migrants from the public health service (unless they paid into a special insurance scheme) had led to a “saturation” of accident and emergency wards in hospitals. Therefore, it does not seem that so much money was saved by the ban (Spanish national-level politician).

Given the severe economic crisis and lack of funds, I found a similar situation in Italy, France and England.

I have been here for almost ten years... now there are more services for immigrants, but often they are not functioning because there is no more money... in general, the situation has worsened (R.K., irregular Nigerian – in Italy).

Moreover, for reasons rooted in social and cultural bias as well, undocumented immigrants are often portrayed as a burden on the state.

Providing basic health care to non-nationals, especially undocumented migrants, will place an extraordinary burden on limited state resources and undermine the sovereign interest of states by “dictating” where to allocate state resources (Italian local politician).

<sup>9</sup> See *The Telegraph*, 20 October 2015. <http://www.telegraph.co.uk/news/worldnews/europe/spain/11509227/>.

## Accessibility

While most European countries provide access only to emergency services, Italy, France and Spain offer greater access to some services or to some categories of undocumented migrants, according to specific laws and regulations.

In France,<sup>10</sup>

Undocumented migrants who do not meet AME (State Medical Assistance) requirements are nevertheless entitled to: care in life-threatening situations; treatment of contagious diseases; all types of health care for children; maternity care; and abortion for medical reasons (French national-level politician).

But, thousands of undocumented migrants who do not meet AME cover criteria, and face other than the abovementioned situations, are dealing with many practical difficulties in getting care.

Unfortunately I do not have AME eligibility, yet in principle I should have access to emergency care, but then in practice, when I go to the hospital, providers always refuse to treat me and sometimes even my children! (K.W., illegal Moroccan – France).

In Spain:

Anyone under 18, regardless of nationality, can access the public healthcare system and be treated in the same manner as a Spanish citizen. This right is extended to pregnant women (regardless of age or nationality) during pregnancy, child birth and post-natal care (female Spanish national-level politician).

<sup>10</sup> See [www.service-public.fr/particulier/vosdroits/F3079](http://www.service-public.fr/particulier/vosdroits/F3079).

In Italy, both citizens and regular foreigners must (it is a right/duty) be enrolled in the National Health Service.<sup>11</sup> But:

Those who do not have or have lost their residence and do not have any ID – typically undocumented immigrants – cannot register with the NHS and be issued with a health card (C.M., female Italian clerk at a health counter).

Consequently:

Those who are not enrolled in the NHS cannot even enjoy the minimum essential levels of health care [the so-called LEA, D.P.C.M. 19 Nov. 2001] (G.L., Italian healthcare assistant).

So we witness a legal vacuum with respect to the requirements of Articles 3 (principle of equality) and 32 of the Italian Constitution (which protects health as a fundamental right of the individual and collective interest).

Yet all interviewed politicians agree with the following statements:

Undocumented immigrants (both irregular and illegal) do have some guarantees of healthcare, namely: access to emergency medical care, surgery and hospital urgent or essential care<sup>12</sup>; even continuing care following an injury and international prophylaxis for infectious diseases which cause a public health hazard (Art. 35 T.U. 1998) (female Italian national-level politician).

What is more, we offer services to all children under 18 and pregnant women, even if undocumented (Italian regional-level politician).

From 2013, under the terms of the Health Social Care Act 2012, a reorganization of the NHS took place in the UK,<sup>13</sup> also designed to reduce numbers entering the country and put pressure on illegal immigrants to leave.

<sup>11</sup> In accordance with Art. 32 Cost. It., Law 833/1978 set up the National Health System. The first specific legislative action on immigration and healthcare was Law n. 943/86; then we have Law n. 39/90; D.L. n. 489/1995; Law 40/98 which with D.lgs. n. 286/98 merged into the T.U. Its Articles 34, 35 and 36 represent a milestone in the healthcare of foreigners. See <http://www.salute.gov.it/>.

<sup>12</sup> Circular n. 5 of 24 March 2000 clarifies the meaning of "urgent care" and "essential care".

<sup>13</sup> [www.nhs.uk](http://www.nhs.uk).

Sometimes, in fact, healthcare policies may be used to combat illegal immigration.

Free NHS care for all migrants will only encourage and attract more people into the country illegally (English local politician).

To give free NHS care to people who have no right to be here is unfair to British citizens and also to immigrants who have arrived here by the correct and legal means (English national-level politician).

Undocumented women and their dependents are especially vulnerable to discrimination.

We [me and my family] are not covered by the NHS and we have no money for care. Last year both my son and the son of a friend of mine [a documented one] had measles. As a matter of fact he was taken care of immediately while my son has not received any care... (M.Y., female undocumented Pakistani – England).

Effectively, the issue that cost must either be covered by the patient or taken out of the hospital budget may create a barrier to care.

A few months ago my husband felt sick, he had contracted Lassa fever (maybe because he works in the sewers), but we still did not realize it was so serious; we went to the emergency department but were given only painkillers. Then it was getting worse and worse, but since we had no money to pay for treatment, the health providers did not want to take care of him. Finally they did, but my family and friends had to make a collection to pay for medical expenses (T.K., female undocumented Pakistani – England).

Access to healthcare may also depend on ignorance about the national legal system as well as fear of detection and deportation.

According to the Italian legal system:

The access to medical facilities by aliens who do not comply with the rules on their stay must not involve any kind of reporting authority, except in cases where it is compulsory to report (Art. 35, c. 5, TU 286/98) (G.L., Italian doctor).

Nonetheless, fear of being reported can lead undocumented people to avoid hospitals and clinics or postpone medical care altogether.

A few months ago I had a bike accident on the way home from work, but I was afraid to go to the hospital and did not go... I was scared! Because in the same circumstances my cousin in Lyon was repatriated (A.G., irregular Nigerian – Italy).<sup>14</sup>

I do not trust them... when you go to the hospital they will give you an identification code<sup>15</sup> which is recognized throughout the national territory... I think that in this way they can easily find and pick you up... (A.H., female illegal Nigerian – Italy).

Here it's really hard to get care when you are ill and even worse it is to go to the hospital. There they ship undocumented migrants back to their home countries after *initial treatment, even without the active involvement of the immigration authorities!* (M.K., undocumented Pakistani – England).

Therefore, undocumented migrants often underutilize the health service compared to natives and legal immigrants.

Since I have no residence permit I never go to the hospital: even if I have to pay a lot, I get care from private physicians illegal or abusive alike (K. N., illegal Nigerian – Italy).

Discrimination looks like an invisible issue because there is no data on it, yet ethnic origin seems to remain the leading reason for healthcare discrimination in relative terms, far ahead of gender.

As soon as they [nurses and doctors at the hospital emergency department] realize that you are an illegal immigrant, and especially coming from Nigeria [there are some Nigerians who do not behave properly, and others who even are in organized crime], they are concerned about helping other people and leave you last (N.K., female illegal Nigerian – Italy).

<sup>14</sup> I could not verify the accuracy of this statement.

<sup>15</sup> It is made up with: the initials STP (Temporarily Present Foreigner), the ISTAT code of the public health service and a sequence number.



According to Olafsdottir's and Bakhtiari's statement (2015, p. 566), many interviewees focus on the notion of "deservingness": undocumented immigrants are often constructed as undeserving of access to social services, including healthcare provisions.

In doing my work I try to help all those who need it, but illegal immigrants, who break the law of our country, do not deserve to be treated like the others [citizenship and regular migrants] (R.P., female Italian healthcare assistant).

Exclusionary discourse, however, is rarely framed in terms of strict legal citizenship. Here citizenship is reframed as not just entailing rights but as tied to obligations.

We cannot help people who do not work and pay social security contributions, social insurance and all other taxes . . . (E.R., French clerk at a health counter).

Because many undocumented migrants are often members of racial or ethnic minority groups, debates inevitably tap into intersecting discourses about race, gender and sexuality.

It is very difficult for us to treat Muslim women: their husbands always want to be present when their wives are examined and they only accept women gynecologists (V.M., Spanish general practitioner).

This ethnic dimension of accessibility overlaps with information and cultural issues and leads us to the third key aspect of the right to health, acceptability.

## Acceptability

Acceptability, understood mainly as respectfulness and cultural appropriateness of the system and healthcare services, inevitably involves taking into account: the immigrants' level of acculturation as well as the lack of knowledge of foreign languages and cultures by health personnel.

Linguistic difficulties can prevent immigrants from accessing health-care in two major ways. First, proficiency in the national language is necessary either to gather information about health services or to effectively communicate medical problems to physicians or eventually to complete health insurance forms. Secondly, without an adequate level of national language proficiency, immigrants are often constrained to jobs which are less likely to provide job-based insurance.

I do not speak Italian, so I cannot get adequate information about how and where to vaccinate my children (A.W., female illegal Nigerian – Italy).

Except the head physician, no one here speaks English fluently; so often we cannot properly understand and help foreigners (P.V., Italian health-care assistant).

Some findings indicate that barriers may exist according to a group's cultural beliefs.

Western health services and care do not conform to our cultural traditions. So I prefer to seek help from a traditional healer who can better understand my pain and sickness" (L.B., female illegal Nigerian – Italy).

Sometimes health providers fail to realize the extreme poverty and marginalization in which illegal immigrants live . . .

We live in an abandoned country house with a lot of damp: every winter the whole family falls sick with coughs and colds . . . often my children also have flu. The doctor said it was my fault because I smoke and we – my wife and me – do not wash, cover, and dress them sufficiently . . . He did not understand our situation . . . (R.W., illegal Pakistani – England).

## Quality

In healthcare, quality is a combination of medical outcomes supported by evidence, safe care and good patient experience.

In theory, each state should move forward in line with the principle of “progressive implementation” of a good-quality, national, public-health service, yet the lack of economic, technical and professional resources make it impossible in practice (Spanish national-level politician).

Speaking about quality issues and obligations of any nation-state, it is important to distinguish between the unwillingness and the inability to do it (French local politician).

Providers sometimes adopt the strategy termed “functional ignorance” (do not ascertain the legal status of a care seeker) (Karl-Trummer et al. 2000), and think that:

To improve the quality to all users without any distinction or discrimination is the responsibility of everyone working in the NHS (S.S. Italian doctor).

Immigrants can also face the providers’ inadequate training.

We are Black and Italian doctors have no idea of our health problems and illnesses (K.Z., illegal Nigerian—Italy).

## Conclusion

Except for a few, limited instances relating to political participation and freedom of movement, the international human rights framework accords full human rights to all migrants without discrimination. Health is a clear example in which the gap between a right and its actual enjoyment is vast. Although undocumented migrants have the right to healthcare under legal conventions adopted by the European Union, my research shows that in most countries this right is interpreted as access to emergency care solely. The situations that fall under the umbrella of emergency care, however, can be different between countries, and the additional strategies to improve access to care can vary along three dimensions: focusing on segments of the population, like children and/or pregnant women; focusing on types of services, like preventive services or treatment of infectious diseases; or using specific funding policies, like

allowing undocumented immigrants to purchase insurance. All this means that non-citizens – undocumented immigrants in particular – receive significantly less healthcare than citizens. According to most respondents, in fact, the purpose of accessibility seems inadequately reached, and everywhere austerity measures threaten the universality, affordability and quality of the healthcare system as well. Due mainly to the global economic crisis and to austerity policies, contemporary health system reforms are especially driven by and result in differential and contingent rights for minority groups, stateless people and undocumented migrants, thereby reinforcing the inequality gaps. Conversely, restrictions based on immigration status are not the only obstacles; there may be barriers related to knowledge, language and culture. My outcomes in fact show that, on the one hand, undocumented immigrants do not perceive health as a basic need, concerned as they are to meet other urgent needs, like food, housing and money. On the other hand, cost-sharing requirements, discriminatory attitudes among providers, the fear of deportation and language and cultural barriers worsen the situation.

Everywhere health policy does not simply reflect the social citizenship boundaries of a country but often directly reinforces legal and social barriers. That is, it encourages a multiplication of differences which are also inequalities. As a result, we witness an increasingly blurred line between citizens and non-citizens (Soysal 1994) and, within the same state, quite different sets of rights for people of different status. Consequently we witness a major trend towards new claims for inclusion, where struggles are framed in the languages of rights and recognition, and hence, of citizenship (Kabeer 2005). Furthermore, the followers of Rousseau’s democratic thought, for whom the citizen ends up taking the place of the natural man, notoriously emphasize the idea according to which the right to citizenship belongs to the rights of the personality, and claim a new form of cosmopolitan citizenship. With few exceptions (Young 1989), many scholars, claiming the principle of “non-discrimination” (Art. 17 EC on Nationality 1997; Art. 13 CE Treaty 1997), argue that the issue of citizenship now belongs to the concept of justice (Benhabib 2004, 2006; Bernts et al. 1992), and insist on the importance of distinguishing between the rights of citizenship (internal and related to a particular political society) and citizenship

right, which is universal: an essential aspect of freedom and personal autonomy and dignity (Howard-Hassmann and Walton-Roberts 2015).

To sum up, the current phase of globalization seems to force us to rethink citizenship in the light of a human rights perspective and thus to elaborate a new model of membership which is tied to universalistic principles rather than country-specific policies (Sassen 2002), and derives its legitimacy from universal personhood, rather than national belonging. Because democracy has an iterative character – it has never been something fixed and unchangeable, but is rather the result of constantly renewed negotiations and reformulations – we can imagine a possible future of peaceful social coexistence only through complex processes of “democratization of democracy” (Balibar 2008). That is, a set of empowering innovative and positive processes, rather than negative moments of resistance, able to put in place acts of transformative and expansive citizenship in the light of solidarity, justice and incorporation (Kivisto and Sciortino 2015).

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# 22

## Dignity and the Invisible Spaces of Irregular Migration: Rendering Asylum Seekers Invisible Through Off-Shore Detention

Claudia Tazreiter

### Introduction

During 2015 the European Union experienced what many media commentators and European politicians called a crisis with close to one million migrants, mostly asylum seekers, arriving through the southern islands and port cities of Italy and Greece. The majority of the migrants were Syrians who had fled the civil war and had come to Europe after spending considerable time in the overcrowded refugee camps that were running short of basic supplies such as food and water. The “crisis” from the perspective of the dominant European debate is a crisis of the arrival of these irregular migrants in such large numbers. Yet at its heart this is a crisis of political will. Though there are many voices in the European debates, a strong thread of continuity voiced early by the German Chancellor, Angela Merkel and later by others has been one of shared responsibility and finding a just solution rather than closing

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